Heart Institute of the Midwest

Dept. of Cardiology

39 Cardiac Way, St. Louis, MO 63101

Discharge Summary

Patient: Otto Braun

Date of Birth: 04/18/1946

Admission Date: 09/15/2024

Discharge Date: 09/23/2024

Attending Physician: Prof. Gerhard Thiele, MD

Dear Colleagues,

Mr. Otto Braun was admitted for decompensated heart failure.

Primary Diagnosis: Heart failure, not otherwise specified (I50.9)

Lab values on admission:

Parameter	Value	Reference
HbA1c	7.9%	< 6.5%
BNP	1240 pg/mL	< 100 pg/mL
Creatinine	1.6 mg/dL	0.7–1.2 mg/dL
Sodium	131 mmol/L	135–145 mmol/L

Mr. Braun was discharged on September 23, 2024 in stabilized condition.

Relevant Medical History:

The patient has a long-standing history of arterial hypertension, first diagnosed in 2008 and currently managed with an ACE inhibitor and a calcium channel blocker. Type 2 diabetes mellitus was diagnosed in 2015 and is managed with oral antidiabetic agents and basal insulin. The patient also has a history of hypercholesterolemia treated with a statin, and underwent cholecystectomy in 2004 without complications. No known drug allergies. The patient reports intermittent use of over-the-counter NSAIDs for chronic joint discomfort, which has been advised against given the renal function.

Family history is significant for coronary artery disease — the patient's father suffered a myocardial infarction at age 62 and underwent coronary artery bypass surgery. The patient's mother was diagnosed with type 2 diabetes at age 70. Two siblings are reported to be in good health. The patient is a former smoker with a 20 pack-year history, having ceased smoking in 2010. Alcohol consumption is moderate, approximately 5 units per week. No illicit drug use was reported.

Physical Examination on Admission:

General appearance: The patient presented in a reduced but stable general condition, awake, oriented, and cooperative. Anthropometric data: height 172 cm, weight 84 kg, BMI 28.4 kg/m2. Vital signs on admission: blood pressure 158/94 mmHg, heart rate 88 bpm and regular, respiratory rate 16 breaths per minute, oxygen saturation 96% on room air, body temperature 37.2°C. Skin was warm, dry, and of normal color with no jaundice, rash, or suspicious lesions noted. Cervical lymph nodes were not palpable.

Cardiovascular system: Heart sounds were regular, with normal S1 and S2. No murmurs, rubs, or gallops detected. Jugular venous pressure was mildly elevated. Peripheral pulses were palpable bilaterally. Respiratory system: Breath sounds were diminished at the right base with mild bibasal crackles on auscultation. No wheezing. Abdomen: soft, non-tender, with no guarding or rigidity. The liver edge was palpable 1 cm below the right costal margin. No splenomegaly. Bowel sounds were present and normoactive. Extremities: 1+ pitting edema present bilaterally below the knees. Neurological examination was grossly intact — cranial nerves II–XII were intact, motor strength was 5/5 in all extremities, and reflexes were symmetric.

Diagnostic Procedures and Results:

12-lead ECG performed on admission: sinus rhythm at 88 bpm, normal axis, no acute ST-segment changes, no T-wave inversions, QTc within normal limits. Transthoracic echocardiography performed on day 2: ejection fraction 58%, preserved systolic function, mild diastolic dysfunction grade I, no significant valvular pathology, no pericardial effusion. Chest X-ray (PA view): no acute pulmonary infiltrate, no pleural effusion, cardiac silhouette within normal limits.

Abdominal ultrasound: liver of normal echogenicity and size, gallbladder status post-cholecystectomy, no biliary dilation, kidneys of normal size with no hydronephrosis, spleen not enlarged, no free abdominal fluid. Duplex sonography of the lower extremities: no evidence of deep vein thrombosis bilaterally. CT thorax and abdomen with intravenous contrast medium: performed on day 3, results reviewed by the radiology department and discussed at the interdisciplinary clinical conference on day 4. Findings are summarized in the separate radiology report appended to this letter.

Medication Prescribed at Discharge:

The following medications were prescribed at the time of discharge, with dosing instructions clearly communicated to the patient: (1) Metformin 1000 mg orally twice daily with meals; (2) Ramipril 5 mg orally once daily in the morning; (3) Atorvastatin 40 mg orally once daily in the evening; (4) Aspirin 100 mg orally once daily; (5) Pantoprazole 40 mg orally once daily before breakfast; (6) Bisoprolol 2.5 mg orally once daily in the morning; (7) Torasemide 5 mg orally once daily. The patient was counseled in detail regarding the purpose, expected benefits, potential side effects, and warning signs for each medication. A written medication plan was provided in the patient's preferred language.

Follow-up Plan and Recommendations:

We recommend the following structured follow-up: (1) Visit to the general practitioner within 2 weeks of discharge for medication review, blood pressure reassessment, and inspection of any wound or access sites. (2) Comprehensive laboratory workup in 3 months including complete blood count, serum electrolytes, renal function panel, liver enzymes, fasting lipid profile, fasting glucose, and HbA1c. (3) Cardiology outpatient follow-up in 6 weeks. (4) Diabetology referral for structured outpatient management optimization. (5) Repeat transthoracic echocardiography in 6 months.

The patient was additionally enrolled in a structured diabetes self-management education program beginning 4 weeks after discharge. Dietary counseling was initiated during the inpatient stay, and a referral to a registered dietitian was arranged for ongoing nutritional support. The patient and accompanying family member were fully informed of the diagnosis, treatment rationale, expected clinical trajectory, and all follow-up requirements. Written discharge instructions were provided, and a direct contact number for the ward team was given in case questions or concerns arise.

Social and Functional Assessment:

Prior to this admission, the patient was living independently at home with a supportive spouse. The patient was fully mobile and independent in all activities of daily living without assistive devices. During the hospital stay, physiotherapy was initiated on day 2 with a focus on mobilization and functional recovery. The patient demonstrated good cooperation and satisfactory progress throughout. Occupational therapy was consulted and confirmed that the patient is safe to return to the home environment without structural modifications.

A comprehensive social work assessment was conducted on day 3 of the hospital stay. No immediate social interventions were identified as necessary at this time. The patient confirmed adequate health insurance coverage and established relationships with a primary care physician and relevant outpatient specialists. Home nursing was arranged to assist with wound care and medication management for a period of two weeks following discharge. Both the patient and spouse expressed confidence in managing the post-discharge plan at home.

Prof. Gerhard Thiele, MD

Director, Heart Institute